PEER REVIEW HISTORY

BMJ Open publishes all reviews undertaken for accepted manuscripts. Reviewers are asked to complete a checklist review form (**http://bmjopen.bmj.com/site/about/resources/checklist.pdf**) and are provided with free text boxes to elaborate on their assessment. These free text comments are reproduced below.

ARTICLE DETAILS

TITLE (PROVISIONAL)	The prevalence and distribution of HPV infection and subtypes in oral squamous cell carcinoma in Africa: A systematic review protocol.
AUTHORS	Holmes, Haly; Afrogeh, Amir; Adeola, Henry; Kimmie-Dhansay, Faheema; Engel, Mark

VERSION 1 – REVIEW

REVIEWER	Hasseus, Bengt University of Gothenburg
REVIEW RETURNED	12-Mar-2021

GENERAL COMMENTS	This protocol is of interest. Little is known about HPV prevalence in Africa, especially south of Sahara. A review that describes knowledge to date and knowledge gaps could be important in initiation of HPV/head and neck cancer studies in Africa. I have two comments: line 165: ..fresh frozen... ? I guess that the authors mean formalin-fixed paraffin embedded specimens-FFPE. References: miss journal names and publication years-makes assessment of ref list difficult- suggest editing here

REVIEWER	Lehtinen, Matti Tampereen Yliopisto, Faculty of Social Sciences
REVIEW RETURNED	25-Mar-2021

GENERAL COMMENTS	The paper by Holmes et al. describes a meticulous attempt for systematic review of HPV-associated oropIharyngeal cancers in Africa. They aim to look carefully for all studies on HPV-positive cancer since 1985, and the Methods section assures the reviewer that this will indeed happen according to the state-of-science. There, however, are problems with regard to the quality and representativeness of the data available in studies published between 1985 - today from the sensitivity/specificity of HPV DNA detection to the accuracy of histopathological diagnoses. There should an apriori plan on how to merge the data most importantly to control misclassification bias. Another problem I am raising in the above points 3, 8 and 12 is the representativeness of the published data (most notably in terms of geography and calendar time). I strongly recommend consulting IARC and WHO data for example to evaluate over time changes in OSCC in differrent parts of Africa. Overall some forms of oropharyngeal cancer (e.g. tonsillar carcinoma) are nowadays OSCC almost exclusively associated with HPV. As for OSCC in Africa increasing over time trends might be disclosed from the avaiable IARC for elaborating

	comparisons with the published HPV-OSCC studies. The impact of HIV/AIDS epidemic in the background (from 1985 onwards!) on occurrence (prevalence and incidence) of OSCC should be considered with regard to pertinent WHO data. Ignoring the possibility to think bigger is a misgiving here.

VERSION 1 – AUTHOR RESPONSE

Reviewer 1: This protocol is of interest. Little is known about HPV prevalence in Africa, especially south of Sahara. A review that describes knowledge to date and knowledge gaps could be important in initiation of HPV/head and neck cancer studies in Africa.	Thank you for the comment and positive sentiment as regards the value of this intended work.
I have two comments: line 165: ..fresh frozen... ? I guess that the authors mean formalin-fixed paraffin embedded specimens-FFPE.	Thank you. Text changed to “formalin-fixed paraffin-embedded...” [Line 165]
References: miss journal names and publication years- makes assessment of ref list difficult- suggest editing here.	Thanks for pointing out the error. Has been updated accordingly.
Reviewer: 2 There, however, are problems with regard to the quality and representativeness of the data available in studies published between 1985 - today from the sensitivity/specificity of HPV DNA detection to the accuracy of histopathological diagnoses.	General comment: In conducting the systematic review, we expect to encounter issues as described by the reviewer. These will be documented and presented so as to give the reader an accurate picture of the current status of HPV-OSCC in Africa. For this review, we have indicated that we will use standardized diagnostic tests as described by the individual authors as an

There should an *a priori* plan on how to merge the data most importantly to control misclassification bias. Another problem I am raising in the above points 3, 8 and 12 is the representativeness of the published data (most notably in terms of geography and calendar time). I strongly recommend consulting IARC and WHO data for example to evaluate over time changes in OSCC in different parts of Africa. Overall some forms of oropharyngeal cancer (e.g. tonsillar carcinoma) are nowadays OSCC almost exclusively associated with HPV. As for OSCC in Africa increasing over time trends might be disclosed from the available IARC for elaborating comparisons with the published HPV-OSCC studies.	entry criterion. We will conduct a subgroup analysis according to diagnostic method used, so as to be able to evaluate the concern of the reviewer. Data extraction will specifically document the site as listed by the authors. Again, we will tabulate these in our results. These databases suggested will be consulted for incorporation in the discussion section
The impact of HIV/AIDS epidemic in the background (from 1985 onwards!) on occurrence (prevalence and incidence) of OSCC should be considered with regard to pertinent WHO data. Ignoring the possibility to think bigger is a misgiving here.	Medical history will be included in the data extraction form. The impact of reporting /non reporting will be included in the discussion